

Anxiety Disorders

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Panic Disorder

Panic attacks:

Panic attacks occur in the context of several different anxiety disorders

Patients with panic disorder report discrete periods of intense terror and fear of impending doom, which are almost intolerable



There are three characteristic types of panic attacks with different relationships between the onset of the attack and the presence or absence of situational triggers:

1. **Unexpected (uncued)**: not associated with situational triggers. These attacks are characteristic of panic disorder
2. **Situationally bound (cued)**: occurs immediately on exposure to situational cue. These are characteristic of specific & social phobias
3. **Situationally predisposed**: more likely to occur on exposure to the situational cue but are not invariably associated with the cue and do not necessarily occur immediately after the exposure. these are specially frequent in panic disorder but may at times occur in specific & social phobias.

DSM-V Criteria for Panic Disorder

A. Recurrent unexpected panic attacks occur, during which **four** of the following symptoms begin abruptly and reach a peak within **10 minutes** in the presence of intense fear:

1. Palpitations, increased heart rate
2. Sweating
3. Trembling or shaking
4. Sensation of shortness of breath
5. Feeling of choking
6. Chest pain or discomfort
7. Nausea or abdominal distress
8. Feeling dizzy, lightheaded or faint
9. Derealization or depersonalization
10. Fear of losing control or going crazy
11. Fear of dying
12. Paresthesias
13. Chills or hot flushes



B. At least one of the attacks has been followed by one month of one of the following:

- a.** Persistent concern about having additional attacks and their consequences, such as; losing control, fear of having a heart attack or going crazy
- b.** A significant maladaptive change in behavior related to the attacks, e.g. avoidance of exercise, or unfamiliar situations.

C. Panic attacks are not due to the effects of a substance or medical condition.

D. The panic attacks are not caused by another mental disorder, such as social phobia, post-traumatic stress disorder.

Clinical Features of Panic Disorder:

- A.** Patients often believe that they have a serious medical condition. Marked anxiety about having future panic attacks (anticipatory anxiety) is common
- B.** In agoraphobia, the most common fears are of being outside alone or of being in crowds or traveling. The first panic attack often occurs without an acute stressor or warning. Later in the disorder, panic attacks may occur in relation to specific situations, and phobic avoidance to these situations can occur
- C.** Major Depression occurs in over 50% of patients. Agoraphobia may develop in patient with simple panic attacks. **Elevation of blood pressure and tachycardia may occur during a panic attack**

Epidemiology of Panic Disorder:

- A.** The lifetime prevalence of panic disorder is between 1.5% and 3.5%. The female-to-male ratio is 3:1. Up to one-half of panic disorder patients have agoraphobia
- B.** Panic disorder usually develops in early adulthood with a peak onset in the mid twenties. Onset after age 45 years is unusual
- C.** First-degree relatives have an 8 fold increase in panic disorder
- D.** The course of the illness is often chronic, but symptoms may wax and wane depending on the presence of stressors. 50% of panic disorder patients are only mildly affected. 20% have marked symptomatology
- E.** The suicide risk is markedly increased, especially in untreated patients. Substance abuse, especially of alcohol, may occur in up to 40% of patients

Differential Diagnosis of Panic Disorder:

- A.** Generalized Anxiety Disorder
- B.** Substance-Induced Anxiety Disorder
- C.** Anxiety Due to a General Medical Condition

Treatment of Panic Disorder:

- A.** Mild cases of panic disorder can be effectively treated with cognitive behavioral psychotherapy with an emphasis on relaxation and instruction on misinterpretation of physiologic symptoms

- B.** Pharmacotherapy is indicated when patients have marked distress from panic attacks or are experiencing impairment in work or social functioning
 - 1.** Serotonin-specific reuptake inhibitors and Tricyclic antidepressants are most often used

 - 2.** SSRIs are the first-line treatment for panic disorder. Initiate treatment at lower doses than used in depression because routine antidepressant doses may actually increase anxiety in panic disorder patients

3. When using a Tricyclic antidepressant, the initial dose should also be low because of the potential for exacerbating panic symptoms early in treatment

4. Benzodiazepines may be used adjunctively with TCAs or SSRIs during the first few weeks of treatment. When a patient has failed other agents, benzodiazepines are very effective
5. Buspirone (BuSpar) is not effective for panic disorder
6. Monoamine oxidase inhibitors (MAOIs) may be the most effective agents available for panic disorder, but these agents are not often used because of concern over hypertensive crisis
7. Medication should be combined with cognitive behavioral therapy for optimal outcome

Separation Anxiety Disorder (SAD)

Abnormal reactivity to real or imagined separation from attachment figures

Interferes with daily activities and development

Lasts longer than 4 weeks

Begin before age 18

Significant distress or impairment

Beyond what is expected at developmental stage

A common psychopathology in youth

Prevalence: 5-25% worldwide

Much lower percentage get treatment

Usually not assessed till refusing school or somatic symptoms

Accounts for half of all anxiety referrals

Different diagnostic criteria than in adults

Negative psychosocial outcomes

Predictive of adult pathology especially panic disorder

Clinical presentation

Heterogeneous

Cardinal symptom = excessive distress or fear with separation from attachment figures

3 characteristics:

- 1) excessive or persistent fears or worries before and at the time of separation
- 2) behavioral and somatic symptoms before, during, and after separation
- 3) persistent avoidance or attempts to escape the separation situation

Worries

- Something happen to parents
 - Disappear
 - Get lost
 - Forget child
- _ Physical safety
 - Something happen to child
 - Get lost
 - Kidnapped
 - Killed

Behaviors

- Crying
- Clinging
- Complaining
- Searching
- Calling

Physical Symptoms

- Headaches
- Abdominal pain
- Fainting, dizziness
- Nightmares, sleep problems
- Nausea, vomiting
- Cramps, muscle aches
- Palpitations, chest pain

Common Triggers

Day care drop-off

Entering school

Boarding school bus

Bedtime

Left at home with babysitter

Summer camp

Moving

Spending the night out

During parental separation or divorce

Change in friends

Bullying

Medical illness

Aetiology

Both biological and environmental factors

Large scale twin study: heritability 73% (Bolton et al, 2006)

Anxiety disorders 5 x more likely if anxious parents

Fear conditioning in fear reward circuits (amygdala, orbitofrontal cortex, anterior cingulate cortex)

Aetiology: Family Risk factors;

Low parental warmth

Little child autonomy

Overprotective and overinvolved parents

Insecure attachment, especially with mother

Anxious mothers

Severe parental discord

Separation or divorce

Parental physical illness

Parental mental illness

Parental job loss

Egocentric, immature, instable or antisocial father

Exposure to family violence

Birth of a sibling

Aetiology: Other Risk factors;

Stressful life events: major disaster or crime

Behavioral inhibition

Low tolerance for humiliation

Fear of failure

Depression

Females > males

Being bullied

Low achievement in school and sports

Differential Diagnosis; rule out medical causes of somatic symptoms;

Anaemia

Infections

Hyperthyroidism

Mitral valve prolapse

Asthma

Gastrointestinal issues

Diabetes mellitus

Lead and mercury levels

Brain tumour

Seizure disorder

BUT AVOID UNNECESSARY INVESTIGATIONS

Treatment

Psychoeducation

Behavioral management

Interventions at school

Interventions at home

Cognitive Behavioral Therapy

Pharmacological treatment

Selective Mutism

A mental disorder where individuals (most commonly children) do not initiate speech or reciprocally respond when spoken to by others. This lack of speech occurs in both social interactions with children or adults. Children will however, speak in their home in the presence of immediate family members.

Children with Selective Mutism are frequently not diagnosed with the disorder until they enter school at around the age of five.

Epidemiology

Selective mutism is rare, and the point prevalence using various clinic or school samples ranges between 0.03% and 1%.

There does not appear to be a sex or ethnicity difference.

It is more likely to occur in young children than adults.

DSM-5 Diagnostic Criteria

- A. Consistent failure to speak in specific social situations in which there is an expectation for speaking (e.g. - at school) despite speaking in other situations.
- B. The disturbance interferes with educational or occupational achievement or with social communication.
- C. The duration of the disturbance is at least 1 month (cannot be during first month of school).
- D. The failure to speak is not attributable to a lack of knowledge of, or comfort with, the spoken language required in the social situation.
- E. The disturbance is not better explained by a communication disorder (e.g. - [childhood-onset fluency disorder](#)) and does not occur exclusively during the course of [autism spectrum disorder](#), [schizophrenia](#), or another [psychotic disorder](#).

Signs and Symptoms

Children with selective mutism often refuse to speak at school, which can cause academic or educational impairment.

The lack of speech can also interfere with social communication. Children may sometimes use non spoken or nonverbal methods (e.g. - grunting, pointing, writing) to communicate.

They may similarly be willing or eager to perform or engage in social encounters when speech is not required (e.g. - nonverbal parts in school plays).

Generally, children with selective mutism have normal language skills

Treatment

Cognitive behavioural therapy/ CBT is the main treatment for selective mutism. Exposure is the most important component, and involves gradually exposing the child to speaking tasks with parental/therapist support.